

REPORT

PT Name :
Ref By :
Reg No :
Barcode :
INV : 17 OH Progesterone

Age : | Sex :
Registered on :
Received on :
Reported on :

SAMPLE COLLECTED AT :
RESONANCE PATHOLOGY LAB

SAMPLE : Serum

17 OH PROGESTERONE

TEST DESCRIPTION	RESULT	UNITS	BIOLOGICAL REFERENCE RANGE
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17 OH Progesterone		ng/mL	. - .
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Method: CLIA

Reference Range

Age	Reference range
< 1 yr	<3.00
>1yr	<2.00
Adult Males	0.63-2.15

Premenopausal females

- Follicular phase 0.32-1.47
- Luteal phase 0.25 – 2.91
- Contraception 0.20 – 1.90
- Postmenopausal Females 0.19 – 0.71

Clinical significance :

The adrenal glands, ovaries, testes, and placenta produce OHPG. It is hydroxylated at the 11 and 21 position to produce cortisol. Deficiency of either 11- or 21-hydroxylase results in decreased cortisol synthesis, and feedback inhibition of adrenocorticotrophic hormone (ACTH) secretion is lost. Consequent increased pituitary release of ACTH increases production of OHPG. But, if 17-alpha-hydroxylase (which allows formation of OHPG from progesterone) or 3-beta-hydroxysteroid dehydrogenase type 2 (which allows formation of 17-hydroxyprogesterone formation from 17-hydroxypregnenolone) are deficient, OHPG levels are low with possible increase in progesterone or pregnenolone respectively.

OHPG is bound to both corticosteroid binding globulin and albumin and total OHPG is measured in this assay. OHPG is converted to pregnanetriol, which is conjugated and excreted in the urine. In all instances, more specific tests are available to diagnose disorders or steroid metabolism than pregnanetriol measurement.

Please correlate with clinical conditions

~~End of report~~